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VIA ELECTRONIC SUBMISSION

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Division of Dockets Management
Department of Health and Human Services
Food and Drug Administration
5630 Fishers Lane, Room 1061 (HFA-305)
Rockville, MD 20852

CITIZEN PETITION

Re: Citizen Petition Requesting Regulatory Action Concerning Compounded Semaglutide Preparations

Dear Sir or Madam:

I am writing on behalf of a client represented by Wiley Rein LLP (“Petitioner”) that respectfully submits this petition under the Federal Food, Drug, and Cosmetic Act (FD&C Act) and Title 21 of the Code of Federal Regulations (C.F.R.) § 10.30 to request that the Commissioner of Food and Drugs take specific and necessary actions to address the significant and escalating public health risks associated with the large-scale production and distribution of compounded drug preparations purporting to contain semaglutide.

A. ACTIONS REQUESTED

Petitioner respectfully requests that the Commissioner of Food and Drugs, in the interest of protecting public health, take the following administrative actions:

1. Issue an order and accompanying guidance clarifying that any Active Pharmaceutical Ingredient (API) intended for use in compounding semaglutide, particularly by facilities registered under Section 503B of the FD&C Act, must be manufactured in a facility that is registered with the FDA and has been inspected and found to be in full compliance with the Current Good Manufacturing Practice (CGMP) requirements set forth in Section 501(a)(2)(B) of the FD&C Act and 21 C.F.R. Parts 210 and 211.
2. Issue an order and/or guidance mandating that for any compounded semaglutide product where the API is produced via a fermentation process—particularly those utilizing the 29-peptide intermediate (P29), CAS No. 1169630-82-3—the compounder or API manufacturer must conduct and submit to the Agency for review a comprehensive immunogenicity risk assessment. This assessment must rigorously evaluate the potential risks arising from process-related impurities,

specifically including host cell protein (HCP) contamination, and product-related impurities, such as peptide aggregates.

USFDA should require: 1) the submission of a separate Drug Master File (DMF) for the fermented intermediate, including but not limited to P29; 2) the inspection and audit of the manufacturing sites involved in the intermediate production; 3) that comprehensive analytical methods be validated to detect impurities, especially those derived from fermentation processes; and 4) a mandate to conduct immunogenicity study and aggregation study on the ANDA utilizing such API with respect to these impurities.

3. Issue an order or guidance classifying compounded semaglutide preparations dispensed to patients in multi-dose vials for self-injection with a manual syringe as “demonstrably difficult to compound” under Section 503A(b)(3)(A) of the FD&C Act. This classification should be based on the high risk of clinically significant dosing errors, which the Agency has already acknowledged have led to adverse events and hospitalizations.
4. Issue new guidance, or reinforce existing guidance, to clarify that injectable drug products intended for subcutaneous (SC) administration, but which are dispensed without an integrated delivery system designed to ensure proper injection depth and prevent inadvertent intradermal (ID) or intramuscular (IM) administration, pose unacceptable risks to safety and efficacy. Such products should be deemed misbranded under Section 502 of the FD&C Act because their labeling cannot ensure they are safe and effective for their intended use.
5. Establish and implement an enhanced, dedicated pharmacovigilance program for high-risk compounded drugs with widespread use, beginning with semaglutide. This program must transcend the limitations of the current FDA Adverse Event Reporting System (FAERS) by incorporating active surveillance methodologies and mandating adverse event reporting from all compounders of these products, including 503A pharmacies, with regular public disclosure of aggregated findings.
6. Issue an order requiring all facilities that compound semaglutide to develop and disseminate a “Dear Health Care Professional” (DHCP) letter, consistent with the requirements of 21 C.F.R. § 200.5. The content of this letter, which must be submitted to the FDA for prior review, shall clearly and conspicuously articulate the multifaceted risks identified in points 1 through 4 of this petition to ensure prescribers can make fully informed clinical decisions.
7. Ensure comprehensive analytical methods are validated to detect impurities, especially those derived from fermentation processes. As part of this requirement, FDA should issue a mandate to conduct immunogenicity study and aggregation studies on the ANDAs utilizing such API with respect to these impurities.

B. STATEMENTS OF GROUNDS

Introduction

A Compounding Crisis Masked as a Convenience: The practice of pharmacy compounding serves a vital role in public health by providing customized medications for patients whose clinical needs cannot be met by FDA-approved drug products.⁵ This traditional practice, however, was intended for individual, patient-specific prescriptions prepared on a small scale. The current market for compounded semaglutide has evolved far beyond this scope, becoming a form of de facto, large-scale manufacturing that systematically bypasses the rigorous safety, efficacy, and quality assurances that are the bedrock of the U.S. drug approval process.⁵

The unprecedented demand for GLP-1 receptor agonists for weight management and diabetes has created a lucrative market for compounders. This demand was initially facilitated by the inclusion of FDA-approved semaglutide products (Ozempic® and Wegovy®) on the FDA’s drug shortage list.⁶ On February 21, 2025, the FDA declared this shortage resolved, a move that legally curtails the ability of compounders to prepare “essentially a copy” of these commercially available drugs.⁶

While this is a necessary and welcome step, it is fundamentally insufficient to protect the public. The resolution of the shortage does not address the millions of doses of compounded semaglutide of unknown quality and safety that are already in the hands of patients, nor does it prevent compounders from creating preparations that are minimally altered to circumvent the “essentially a copy” rule. The underlying risks inherent to these products—risks spanning API quality, immunogenicity, and unsafe administration methods—persist and demand direct, forceful regulatory action. These compounded preparations are not equivalent to their FDA-approved counterparts, and the chasm in their risk profiles, as summarized in Table 1, represents a clear and present danger to public health.

Table 1: Comparative Risk Profile: FDA-Approved Semaglutide vs. Compounded Preparations

Attribute	FDA-Approved Semaglutide (e.g., Ozempic®, Wegovy®)	Compounded Semaglutide Preparations
API Source & Quality	Sourced from FDA-registered facilities inspected for compliance with Current Good Manufacturing Practice (CGMP). ¹¹	API source is often unknown, unverified, and may not be from an FDA-registered or inspected facility. 503A facilities are not subject to federal CGMP requirements. ⁵
Immunogenicity Profile	Clinically characterized immunogenicity profile	Unknown immunogenicity risk. If API is from fermentation,

	based on data from thousands of patients. Manufacturing process controlled to minimize impurities. ¹⁴	potential for contamination with host cell proteins (HCPs) and aggregates, which can act as adjuvants and increase immunogenicity, exists without any required testing. ¹⁵
Delivery System & Dosing Accuracy	Provided in a single-patient-use, pre-filled autoinjector pen designed and validated for precise, fixed-dose delivery. ¹⁷	Typically dispensed in multi-dose vials requiring manual measurement with a syringe, a method the FDA has linked to dosing errors and hospitalizations. ¹⁹
Administration Route Assurance	The autoinjector's fixed needle length and automated injection mechanism are engineered to ensure consistent subcutaneous (SC) delivery. ²¹	Manual injection with a syringe lacks controls for needle depth, creating a significant risk of inadvertent and improper intramuscular (IM) or intradermal (ID) administration, altering pharmacokinetics and safety. ²²
Pharmacovigilance	Mandatory post-market adverse event (AE) reporting by the manufacturer to the FDA Adverse Event Reporting System (FAERS). ²⁵	No federal requirement for 503A pharmacies to report AEs to the FDA, resulting in what the Agency calls likely "underreported" adverse events and a critical public health blind spot. ¹⁹
Risk Communication	Risks are communicated through FDA-approved labeling and, when necessary, through mandated programs like Risk Evaluation and Mitigation Strategies (REMS) or DHCP letters. ²⁶	No requirement for compounders to proactively inform prescribers of the unique and significant risks associated with their unapproved products, creating a dangerous information asymmetry.

B.1. Unregulated API Sourcing and the Violation of CGMP Standards Present a Foundational Risk to Patient Safety:

The quality, purity, and safety of any finished drug product are inextricably linked to the quality of its Active Pharmaceutical Ingredient (API). The FDA's CGMP regulations, codified in 21 C.F.R. §§210 and 211, establish the minimum requirements for the methods, facilities, and

controls used in drug manufacturing to assure that a drug product is safe for use and has the identity, strength, quality, and purity it purports to possess.¹¹ These regulations are not arbitrary; they are the fundamental safeguards against contamination, impurities, and manufacturing variability that can lead to patient harm. Section 501(a)(2)(B) of the FD&C Act explicitly states that a drug is deemed adulterated if the methods used in its manufacture, processing, packing, or holding do not conform to CGMP.¹²

A critical vulnerability in the current regulatory landscape for compounded drugs lies in the differential application of these standards. While outsourcing facilities registered under Section 503B are subject to CGMP requirements and routine FDA inspection, traditional state-licensed pharmacies operating under Section 503A are exempt from federal CGMP compliance.⁵ This distinction, intended for pharmacists preparing bespoke medications for individual patients, has been exploited by the industrial-scale demand for semaglutide. A significant portion of the estimated 80 million compounded semaglutide prescriptions are filled by 503A pharmacies, creating a market where drugs are produced on a massive scale but are governed by quality standards designed for small-batch preparations.⁸

This regulatory gap is profoundly dangerous. The FDA itself has repeatedly warned about the risks associated with API from unknown or unregulated suppliers. The Agency has urged compounders to know their suppliers and ensure they receive valid certificates of analysis from FDA-registered facilities.¹³ These warnings are based on documented instances of API repackagers distributing mislabeled or contaminated substances, creating serious vulnerabilities in the drug supply chain.¹³ This risk is not theoretical; it is a documented reality. In the context of semaglutide, this concern is amplified by reports from state attorneys general and the FDA regarding counterfeit products and the illegal importation of semaglutide API from foreign sources, such as China, whose manufacturing standards and supply chain integrity are entirely unknown and unverified.¹⁰ Novo Nordisk has noted that a majority of Chinese manufacturers exporting “semaglutide” API are not even permitted to distribute that API for human use within China, a fact that underscores the grave risk to U.S. patients.²⁹

For a potent, systemically acting peptide drug like semaglutide, administered long-term to millions of patients for chronic conditions, the sourcing of its API cannot be a regulatory afterthought. Allowing the foundational component of this therapy to be sourced from facilities that have not been inspected by the FDA for CGMP compliance represents an unacceptable gamble with public health. The Agency must therefore act to close this loophole and require that any API used for compounding semaglutide, especially at the scale seen in 503B facilities, meets the same baseline quality standard—CGMP compliance—as the API used for FDA-approved medicines.

B.2. The Unquantified Immunogenic Potential of Fermentation-Derived Semaglutide and its Impurities

The safety of a therapeutic peptide is not solely a function of its primary amino acid sequence; it is also critically dependent on its purity and the absence of immunogenic contaminants. The manufacturing process itself is a key determinant of this risk profile. While some semaglutide API may be produced via full solid-phase chemical synthesis, a common and cost-effective

method involves the use of recombinant DNA technology.³⁰ This process often utilizes the 29-peptide backbone of semaglutide, known as P29 or GLP-1(9-37) (CAS No. 1169630-82-3), as a key intermediate.³⁰ This P29 intermediate is frequently produced through fermentation in a host organism, such as the bacterium *Escherichia coli* (*E. coli*).³¹

This choice of manufacturing pathway introduces a specific and significant class of risk that is absent in chemically synthesized peptides: immunogenicity driven by process- and product-related impurities. Unwanted immune responses to peptide drugs can neutralize their therapeutic effect, alter their pharmacokinetics, and cause serious adverse events, including life-threatening anaphylaxis.³⁴

Two categories of impurities are of paramount concern for fermentation-derived semaglutide:

1. **Process-Related Impurities (Host Cell Proteins):** When peptides are produced in a biological system like *E. coli*, it is impossible to achieve 100% purity. Trace amounts of proteins from the host organism, known as Host Cell Proteins (HCPs), inevitably co-purify with the final drug substance.³⁶ These HCPs are not inert bystanders. Scientific research has demonstrated that certain HCPs can act as powerful adjuvants, meaning they can stimulate and amplify the patient's immune response to the therapeutic peptide itself.¹⁶ A key example is the *E. coli* chaperone protein DnaK (a 70 kDa heat shock protein), which has been identified as a common contaminant in recombinant protein preparations and has been shown to possess significant adjuvant-like activity, particularly when associated with aggregated forms of the therapeutic protein.¹⁶ The presence of such HCPs transforms the drug product from a simple peptide into a complex biologic formulation with an uncharacterized and potentially enhanced immunogenic potential.
2. **Product-Related Impurities (Aggregates):** During manufacturing, purification, and storage, peptide molecules can self-associate to form aggregates. These aggregates are widely recognized as being significantly more immunogenic than their monomeric counterparts.¹⁶ They can present repetitive epitopes to the immune system, leading to B-cell activation and the formation of anti-drug antibodies (ADAs). The presence of HCPs can exacerbate this risk, as they may associate with the aggregated peptide, creating a highly immunogenic complex.¹⁶ Furthermore, the P29 intermediate itself can be a source of product-related impurities, such as deletion or insertion variants, if the synthesis and purification are not exquisitely controlled.³⁸

The FDA's own thinking on the regulation of generic peptides has evolved to recognize the critical importance of impurities. Agency guidance now recommends that applicants for generic peptides conduct a thorough risk assessment of any new peptide-related impurities present at levels between 0.1% and 0.5%.¹⁵ Yet, under the current compounding framework, semaglutide derived from fermentation—containing a complex and uncharacterized mixture of potential HCPs and aggregates—is being injected into patients without any such risk assessment. This is a profound inconsistency in regulatory standards. A robust control strategy—comprising rigorous purification, orthogonal analytical methods, and alignment with FDA and ICH guidelines—is

essential to mitigate these risks and ensure product safety, efficacy, and consistency. A compounded product made from fermentation-derived API is not merely a copy of the approved drug; it is a fundamentally different formulation containing known immuno-stimulatory contaminants. This is not a theoretical risk but a predictable consequence of the manufacturing method. The Agency must therefore require that any compounder using such API perform the same rigorous immunogenicity risk assessment that would be demanded of any generic or follow-on biologic manufacturer.

B.3. The Ultimate Immunogenic Risk: Triggering Autoimmunity Against Natural GLP-1

Beyond the risks of reduced efficacy or localized allergic reactions, the unchecked immunogenicity of poorly-controlled compounded semaglutide presents a far more catastrophic possibility: the induction of a widespread, irreversible autoimmune response against the body's own natural glucagon-like peptide-1 (GLP-1). This potential for a public health tragedy stems from a well-established immunological phenomenon known as molecular mimicry.

Molecular mimicry occurs when the immune system, after being primed to attack a foreign antigen (such as a peptide aggregate or host cell protein impurity), mistakenly attacks a structurally similar self-protein. Semaglutide is a GLP-1 analog, sharing 94% sequence homology with the native human hormone.⁴¹ This high degree of similarity is precisely what makes it therapeutically effective, but it also creates the perfect conditions for molecular mimicry if the immune system is improperly stimulated.

The danger lies with the uncontrolled impurities present in some compounded preparations. Aggregates and residual host cell proteins (HCPs) from fermentation can act as powerful adjuvants, provoking a strong antibody response where one might not otherwise occur.¹⁶ If the immune system generates antibodies against a complex of semaglutide and these impurities, it may learn to recognize the semaglutide molecule itself as a threat. Due to the structural similarity, these anti-drug antibodies (ADAs) could then cross-react with and neutralize the body's endogenous GLP-1.

This is not a theoretical concern. Clinical studies of other GLP-1 receptor agonists have documented this exact outcome. For instance, in studies of dulaglutide, another GLP-1 analog, 1.6% of patients developed ADAs. Of those, a significant portion (0.9% of the total population) had ADAs that cross-reacted with native GLP-1, and a smaller but critically important group (0.1% of the total population) developed antibodies that actively neutralized their own natural GLP-1.⁴² Furthermore, large-scale real-world evidence has already linked the use of GLP-1 receptor agonists to an increased risk of developing autoimmune diseases, such as autoimmune thyroiditis and rheumatoid arthritis.⁴³

The consequences of inducing a widespread autoimmune response against natural GLP-1 would be devastating. Endogenous GLP-1 is a critical hormone for maintaining glucose homeostasis, regulating insulin and glucagon secretion, and influencing cardiovascular and digestive systems. An autoimmune attack could permanently disable these vital functions, leading to intractable metabolic disorders in a large population of American patients. Exposing millions of individuals

to uncontrolled products with the potential to trigger such a response, without any of the immunogenicity risk assessment required for approved drugs, constitutes an unacceptable and potentially uncontrollable public health gamble.

B.4. Uncharacterized Container Interactions: The Risks of Glass Vials in Compounding

The safety and stability of an injectable drug are not determined by the API and excipients alone; they are also critically dependent on the primary container closure system. For FDA-approved drugs, this system is a rigorously tested component, with extensive studies performed to ensure that the container—typically a Type I borosilicate glass vial—does not adversely interact with the drug product over its shelf life.⁵¹ Compounded drugs, however, are exempt from these requirements, introducing another significant and unquantified risk to patients from drug-container interactions.⁵¹

Type I borosilicate glass is the industry standard for parenteral drugs due to its high chemical resistance.⁵² However, it is not inert. The manufacturing process used to convert glass tubing into vials involves high heat, which can cause volatile components like boron and sodium to evaporate and re-condense non-uniformly on the inner vial surface.⁵³ This creates chemically heterogeneous areas that are less durable and more susceptible to chemical attack from the drug solution, leading to two major problems:

1. **Leaching and Delamination:** Over time, elements from the glass matrix—including aluminum, boron, sodium, and silicon—can leach into the drug solution.⁵⁴ In severe cases, this chemical corrosion can cause thin, flexible glass flakes (lamellae) to detach from the vial surface, a phenomenon known as delamination. The presence of these subvisible particles poses a direct risk to patients and has been the cause of numerous product recalls for FDA-approved drugs.⁵⁵ For compounded semaglutide, where no such quality control is mandated, the risk of injecting glass particles is entirely unmonitored.
2. **Metal-Catalyzed Degradation:** The leaching of metallic impurities is particularly concerning for peptide drugs like semaglutide. Trace metals, such as iron and aluminum, can act as catalysts that accelerate the oxidative degradation and aggregation of the peptide.⁵⁶ This leads to the formation of high molecular weight proteins (HMWPs), which reduce the drug's potency and, as previously discussed, significantly increase its immunogenic potential.⁵⁶ Studies of follow-on semaglutide products have confirmed the presence of trace metal impurities not found in the originator drug, highlighting the variability that arises from different manufacturing and packaging processes.⁵⁷
3. **Adsorption and Potency Loss:** Beyond chemical interactions, peptides are known to physically adsorb to the negatively charged surface of borosilicate glass.⁵⁵ This process can remove a substantial amount of the active drug from the solution, leading to unpredictable and sub-potent dosing. One analysis of semaglutide stored

in vials noted that drug recovery was “greatly reduced” after only six hours, indicating a significant and rapid loss of active ingredient to the container surface.

For an FDA-approved product, these potential interactions are thoroughly investigated through mandatory extractable, leachable, and long-term stability studies. For compounded semaglutide, no such studies are required. Patients are therefore exposed to a product whose final purity, potency, and safety profile are unknown, not only because of the unverified API but also because of uncharacterized interactions with its container.

B.5. & B.6. The Dual Dangers of Manual Administration: Dosing Inaccuracy and Improper Injection Depth

The FDA-approved semaglutide products, Ozempic® and Wegovy®, are not simply vials of liquid; they are sophisticated drug-device combination products. The medicine is provided in a pre-filled, single-patient-use autoinjector pen. This device is not a matter of convenience; it is an integral and critical component of the product’s safety and efficacy profile, engineered to mitigate two of the most significant risks associated with self-administered injectable drugs: incorrect dosing and improper injection depth. The widespread practice of dispensing compounded semaglutide in multi-dose vials for manual injection with a standard insulin-type syringe strips away these essential safety features, exposing patients to a dual threat of harm.

First, the risk of dosing error is acute. The FDA has already received “multiple reports of adverse events, some requiring hospitalization, that may be related to dosing errors associated with compounded injectable semaglutide products”.¹⁹ The Agency’s own guidance acknowledges that pen injectors are designed to “enhance safety, improve dosing accuracy, and increase patient compliance”.¹⁷ Requiring a patient, who may be elderly or have dexterity issues to accurately draw a very small volume of a potent medication (e.g., 0.25 mg or 0.5 mg) from a multi-dose vial is fraught with peril. The potential for under-dosing, leading to a lack of efficacy, or, more dangerously, over-dosing, leading to severe gastrointestinal events or other toxicities, is unacceptably high.²⁰ The autoinjector’s “click-to-dose” mechanism virtually eliminates this risk by delivering a precise, pre-set, and validated dose with each use.

Second, the lack of an engineered delivery system creates a significant risk of administration into the wrong tissue compartment. Semaglutide is specifically formulated and approved for subcutaneous (SC) injection.²⁴ The fatty SC tissue has a relatively limited blood supply, which allows for the slow, steady absorption required to achieve the drug’s long half-life and once-weekly dosing profile.²³ Manual injection with a syringe of variable needle length, administered by a patient without medical training, introduces a high probability of error in injection depth.

1. **Inadvertent Intramuscular (IM) Injection:** If the needle is inserted too deeply, the drug enters the muscle tissue. Muscle is highly vascularized compared to SC tissue, which can lead to much more rapid and erratic drug absorption.²¹ This would fundamentally alter the drug’s PK profile, potentially causing a rapid spike in concentration and increasing the risk of acute adverse events. Furthermore, IM injections carry their own risks, including localized pain, hematoma, and, in rare cases, nerve or muscle damage.²²

2. **Inadvertent Intradermal (ID) Injection:** If the injection is too shallow, the drug is delivered into the dermis. The dermis is rich in antigen-presenting cells, such as dendritic cells and Langerhans cells.⁴⁴ Delivering a peptide drug to this immunologically active environment can significantly increase the risk of an immunogenic response, including the formation of ADAs that could neutralize the drug or cause hypersensitivity reactions. The absorption from the ID space is also highly variable and unpredictable.²³

The autoinjector for the approved products is designed with a specific, fixed needle length and an automated injection process to ensure the needle reliably reaches the SC space and not the muscle or dermis.²¹ By providing semaglutide as a drug in a vial, compounders are divorcing the active ingredient from its essential delivery technology. This renders the product's labeling, which is predicated on SC administration, effectively meaningless, as there is no assurance the patient can achieve it. Such a product, which cannot be reliably delivered in the correct dose and to the correct tissue, is inherently unsafe and should be considered misbranded.

B.7. A Call for Modernized Pharmacovigilance: The Blind Spot of Compounded Drug Adverse Events

Pharmacovigilance (PV), the science and activities relating to the detection, assessment, understanding, and prevention of adverse effects, is a cornerstone of modern drug safety.⁴⁶ For FDA-approved drugs, a robust system exists, centered on the FDA Adverse Event Reporting System (FAERS), to which manufacturers are legally mandated to report all serious and unexpected adverse drug experiences.²⁷ While this passive surveillance system has its own well-documented limitations—such as incomplete or duplicate reports and the inability to establish causation or true incidence rates—it provides an essential, albeit imperfect, safety net.²⁵

For compounded drugs, this safety net is riddled with holes. The most significant of these is a structural flaw in the reporting requirements: while 503B outsourcing facilities are required to report serious adverse events to the FDA, the vast majority of compounders operating as 503A state-licensed pharmacies have no such federal mandate.²⁷ The FDA has explicitly acknowledged this dangerous gap, stating that it is “likely that adverse events from compounded versions of these drugs are underreported”.¹⁹ As of July 2025, the FDA had received over 600 adverse event reports for compounded semaglutide, but this number almost certainly represents a small number of the adverse events that occurred.¹⁹

To have a product used by millions of patients, with the myriad risks detailed in this petition, subject to what is effectively a voluntary and fragmented safety monitoring system is a failure of public health policy. The scale of use and the magnitude of risk associated with compounded semaglutide demand a surveillance system commensurate with the threat. The FDA has already developed and embraced more sophisticated, proactive PV tools for approved drugs, and these models should be adapted for high-risk compounded products. Examples include:

1. **The Sentinel Initiative:** This program actively monitors the electronic health records and insurance claims data of over 300 million people to evaluate drug safety risks in a real-world setting.²⁶ An active surveillance approach is desperately

needed to understand the true incidence of adverse events from compounded semaglutide.

2. **Targeted Pharmacovigilance Strategies (PVS):** For drugs with known safety concerns, such as medication errors associated with the monoclonal antibody nirsevimab, the FDA has developed specific PVS plans that involve more frequent and focused evaluation of FAERS and VAERS data.²⁶ A similar targeted strategy is warranted for compounded semaglutide.
3. **Risk Evaluation and Mitigation Strategies (REMS):** For products with serious safety concerns, the FDA can require a REMS, which is a drug safety program designed to ensure a medicine's benefits outweigh its risks.²⁶ Elements of a REMS, such as mandatory provider education or patient monitoring, could be adapted to the compounding context.

It is illogical and indefensible to apply the *least* rigorous surveillance standard to the *most* risky and poorly characterized products. The current system creates a massive public health blind spot. The FDA must act to remedy this by establishing a dedicated, enhanced pharmacovigilance program for compounded semaglutide and other high-risk compounded drugs, one that mandates reporting from all compounders and employs modern, active surveillance methods to truly protect the public.

B.8. The Mandate for Proactive Risk Communication to Health Care Professionals

The final, critical point of failure in the current system is the profound information asymmetry between the compounders who create these high-risk products and the healthcare professionals who prescribe them. It is unreasonable to expect a busy clinician to be an expert in API sourcing, peptide immunogenicity, or the nuances of subcutaneous drug delivery systems. Prescribers, acting in good faith to provide patients with a more affordable or accessible option, are likely operating under the dangerously false assumption that compounded semaglutide is simply a generic-like version of the FDA-approved product.

As this petition has exhaustively detailed, this is not the case. The confluence of risks—from uninspected API, potential immunogenic contaminants, and the high potential for dosing and administration errors—constitutes a “significant hazard to health”.²⁸ This is precisely the scenario for which the FDA’s regulations on “Dear Health Care Professional” (DHCP) letters were designed.

Under 21 C.F.R. § 200.5, the FDA has the authority to require manufacturers to issue DHCP letters to convey critical safety information. Specifically, an “Important Drug Warning” letter is appropriate when there is “important new safety information...that concerns a significant hazard to health” that could affect a provider’s decision to use a drug or change their clinical management.²⁸ The accumulated evidence regarding compounded semaglutide unequivocally meets this standard.

Therefore, the petitioner requests that the FDA mandate that all facilities compounding semaglutide disseminate a DHCP letter to every prescribing practitioner. This letter is not a promotional tool; it is an essential risk communication vehicle. Consistent with FDA guidance, the content of the DHCP letter must be reviewed by the Agency and should clearly and concisely summarize the following:²⁸

- The product is not FDA-approved and has not been reviewed for safety, quality, or efficacy.
- The risks associated with API from unverified sources not subject to CGMP.
- The potential for heightened immunogenicity if the API is derived from fermentation.
- The documented risk of dosing errors and the importance of proper administration technique.

This action represents a necessary, immediate measure to arm prescribers with the information they need to protect their patients while the FDA deliberates on the more systemic regulatory changes requested in this petition. It is the Agency's duty to ensure that the individuals making clinical decisions are not doing so without understanding the true risks of the products they prescribe.

C. ENVIRONMENTAL IMPACT

Petitioner claims a categorical exclusion from the requirement to prepare an environmental assessment under 21 C.F.R. § 25.30, § 25.31, or § 25.32. To Petitioner's knowledge, no extraordinary circumstances exist that would prevent this claim.

D. ECONOMIC IMPACT

Information on the economic impact of the requested actions is not required for submission of this petition. A statement on economic impact will be submitted only upon specific request from the Commissioner of Food and Drugs, in accordance with 21 C.F.R. § 10.30(b).

CERTIFICATION

The undersigned certifies that, to the best knowledge and belief of the undersigned, this petition includes all information and views on which the petition relies, and that it includes representative data and information known to the petitioner which are unfavorable to the petition. Sincerely,

A handwritten signature in black ink, appearing to read "Neal Seth", with a horizontal line extending to the right from the end of the signature.

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REFERENCES

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